DPP	DEPARTMENT POLICY AND PROCEDURE		
Title	**CHEST PHYSIOTHERAPY**		
Department	**Pediatric Intensive Care Unit**		
Applies to	**All PICU Medical and Nursing Staff**		
Policy Code and Version Number	**DPP-PICU-009 (v3)**		
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1.0 PURPOSE

1.1. To loosen and mobilize secretions.

1.2. To reduced effectiveness of cough.

1.3. To unify the care of patient needs chest physiotherapy.

1.4. To eliminate secretions, re-expand lung tissue and promote efficient use of respiratory muscles.

2.0 DEFINITION/ABBREVIATION

2.1. **Chest Physiotherapy (CPT)** - is a technique used to mobilize and loose secretions in lungs and respiratory tract by physical means. It is used to assist a cough, re-educate breathing muscles and to try to improve ventilation of the lungs.

3.0 RESPONSIBILITY

3.1 **Physician** must be able to specify if postural drainage and vibration can be safely administered and specify the specific areas of lungs are to be treated.

3.2 **Head nurse** is responsible to make sure that all needed equipment are available in the unit and staff nurses are well competent in providing CPT nursing care.

3.3 **Charge Nurse is r**esponsible to double check if orders are carried out properly and update the kardex.

3.4 **Staff Nurses** are responsible to carry out physician's orders, maintain proper documentation.

3.5 **Respiratory therapist** is responsible to provide CPT to patient.

4.0 POLICY

4.1. Chest physiotherapy includes postural drainage, chest percussion vibration and coughing with deep breathing exercises.

4.2. Chest physiotherapy must be followed by suctioning if patient has ineffective cough reflex.

4.3. Suction equipment and supplemental oxygen must be ready at bedside.

4.4. Chest physiotherapy must be done by a certified nurse or respiratory therapist upon doctor's order.

4.5. Ensure adequate hydration to patient receiving chest physiotherapy.

5.0 PROCEDURE (S)

5.1. **Preparatory phase**

5.1.1. Identify the correct patient using IPSG goal 1.

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5.1.2. Wash and dry hand thoroughly. To prevent spread of infection.

5.1.3. Gather the equipment at the patient bedside.

5.1.4. Explain the procedure to the patient.

5.1.5. Auscultate the patient's lung before the procedure.

5.1.6. Position accordingly to the area of the lungs to be drained:

5.1.7. In general disease, drainage usually begins with lower lobes, continues the middle lobes and ends with the upper lobes.

5.1.8. In localizes disease, drainage begins with the affected lobes and then proceed to the other lobes.

5.2. **Performance phase**

5.2.1. Instruct the patient to remain in each position for 10 to 15 minutes. And perform percussion and vibration.

5.2.2. To performed percussion, instruct to inhale slowly and deeply using the diaphragm. Percuss with cupped hands over the chest wallover each segment for 1 to 2minutes by alternating your hands against the patient in a rhythmic manner. Listen to hollow sounds on percussion.

5.2.3. To performed vibration, ask the patient to exhale deeply and then exhale slowly through pursed lips. During exhalation, pressed firmly your fingers and palms of the hands against the chest wall. It helps dislodge mucus plugs and mobilize secretions towards the main bronchi and trachea.

5.2.4. Avoid clapping over the spine, liver, kidney, spleen scapula, sternum and clavicle.

5.2.5. After postural drainage, percussion and vibration, instruct the patient to cough or do suction to patient who has no effective cough reflex.

5.2.6. Provide oral hygiene.

5.2.7. Listen to the stethoscope for changes in breath sounds.

5.2.8. Initiate supplemental oxygenation if needed.

5.3. **Follow up phase**

5.3.1. The date and time of chest physiotherapy

5.3.2. Position for secretion drainage

5.3.3. Length of each time maintained

5.3.4. Color, amount, odor and viscosity of secretions produced

5.3.5. Any complication noted along the procedure

6.0 EQUIPMENTS

6.1. None

7.0 ATTACHMENT/FORMS

7.1. N/A

8.0 REFERENCES

8.1. MOH Guidelines for Pediatric Care

8.2. DPP-PICU-0010(V2) – Chest Physiotherapy – PMSH, 15/02/1439

8.3. Saudi Central Board for Accreditation of Healthcare Institution (CBAHI) PICU standards, 3rd version, January 2016

9.0 REVISION CLAUSE

9.1. Used the new logo for AL-Baha Health Cluster.

9.2. Modify the policy and procedure lay out, changed the content of the Header and footer

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